

ENGLISH FOR WORK / SEPTEMBER 2026

Medical Laboratory Technician English



Learner workbook

Work with realistic cases, develop your own response, and use the answer section after you have tried the tasks.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Medical laboratory technicians and clinical laboratory support staff.

All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

01 Accessioning and unambiguous identifiers

Clarify similar laboratory identifiers without merging cases.

02 Specimen suitability and recollection language

Explain suitability wording without interpreting health status or teaching collection.

03 Quality-control status and reportable results

Explain why a completed run is not necessarily a released report.

04 Reagents, instruments, and service coordination

Coordinate equipment status without giving technical operating instructions.

05 Turnaround times and realistic updates

Explain laboratory timing without inventing a guaranteed deadline.

06 Preliminary, final, and amended reports

Repair an inaccurate version statement without interpreting results.

07 Critical notifications and acknowledged handoffs

Communicate a fictional result accurately and confirm its receipt by the stated recipient.

08 Referral laboratories and outstanding work

Give a send-out handoff with a clear remaining task and accepted owner.

Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop, or use the prompts at the end of this guide.

LESSON 01 / READ AND NOTICE

Accessioning and unambiguous identifiers

Clarify similar laboratory identifiers without merging cases.

SITUATION

The receiving log lists accession A314; a clinic caller asks about A341. No identity match has been established. Technician Noor can check the accession log. The caller must identify the requested case through the laboratory's privacy and verification arrangements.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

accession number - A laboratory-assigned identifier linking a received specimen or case to its testing records.

requisition - The clinician's paper or electronic request identifying the laboratory tests ordered.

specimen label - The identifying information attached to a specimen container under the laboratory's requirements.

accessioning - The laboratory's process of registering a specimen or case and associating it with its order.

Useful word combinations

confirm the accession number: establish which laboratory case is being discussed

match the requisition to the label: compare the order and specimen identifiers

read back an identifier: repeat a complete code to check accurate communication

report an accessioning discrepancy: identify an unresolved registration mismatch

LESSON 01 / LANGUAGE AND PRACTICE

Resolve the reference before answering the status question

3. Notice the language

Repeat the complete identifier and name its source. A similar name, location, or code is not a verified match. These exchanges do not replace the laboratory's identity requirements.

Your enquiry says ____; the receiving log lists ____.

Could you read the complete ____ back?

We have not yet established that ____ refers to ____.

Compare this wording: The digits are nearly the same, so it must be your sample.

Clearer: A314 and A341 differ. Let's establish the intended accession before discussing status.

The revised wording prevents a reference guess from becoming an identity claim.

4. Check the language

1. Clinic: Which reference did the laboratory assign to this received case?

Technician: Its ____.

A. accession number

B. invoice total

C. reagent code

D. appointment date

2. Caller: A341 is the same as A314, isn't it?

Technician: No. The digits are ____, so we must clarify the reference.

A. transposed

B. omitted

C. identical

D. consecutive

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 01 / 05 • CONVERSATIONS / 1 OF 3

A314 or A341

Fictional laboratory call using the accession case.

01 • Clinic caller: I'm asking about A341. Has it reached the laboratory, or is it still with the courier?

02 • Technician: Our log lists accession number A314. Let's clarify it before discussing specimen status.

03 • Clinic caller: Both came from our clinic, so I assumed the two references described the same case.

04 • Technician: The clinic location doesn't establish a match. Could you check the reference on your request?

05 • Clinic caller: The requisition says A314. I transposed the last two digits in my note.

06 • Technician: Thank you. Please repeat the full identifier so we can confirm the enquiry accurately.

07 • Clinic caller: A314. I'm asking whether that specimen's receipt is recorded, not whether a result is complete.

08 • Technician: Following our verification checks, I can confirm receipt for A314. Testing status is a separate question.

09 • Clinic caller: I'll correct my enquiry note to A314 and keep the question limited to receipt.

10 • Technician: That resolves the reference mismatch. Please use A314 for any further enquiry about this case.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 2 OF 3

An order without a registered specimen

Fictional electronic order exists, but laboratory receipt has not been confirmed.

01 • Clinic caller: The order appears in our system, so why can't I see an accession receipt?

02 • Technician: An electronic order and a received specimen are different things. I can see the request, not confirmed receipt.

03 • Clinic caller: Does that mean the specimen is lost somewhere between the clinic and the laboratory?

04 • Technician: We haven't established that. The order alone doesn't tell us whether collection or dispatch occurred.

05 • Clinic caller: I'll check the clinic's collection and courier records instead of assuming the laboratory has it.

06 • Technician: Please use the order reference when you check. That helps us compare the same enquiry.

07 • Clinic caller: Can you mark received while we look, just so the order stays visible to staff?

08 • Technician: Received would be inaccurate. The order can remain visible with specimen receipt unconfirmed.

09 • Clinic caller: Understood. I'll report that distinction to the clinician rather than say testing has begun.

10 • Technician: I'll keep the receipt enquiry open under the order reference while your clinic checks its records.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 05 • CONVERSATIONS / 3 OF 3

One request, an additional container

Fictional receiving enquiry about a second container; intended relationship is unresolved.

01 • Technician: A second container arrived with paperwork mentioning the same request. Was it intended as an additional specimen?

02 • Clinic coordinator: I need to check with the collection team. The courier sheet doesn't explain the relationship.

03 • Technician: Please don't infer that it replaces the first specimen simply because it arrived later.

04 • Clinic coordinator: I won't. Do you need the intended relationship or confirmation of a particular test result?

05 • Technician: The relationship is the question. No result status is being discussed in this call.

06 • Clinic coordinator: I'll ask the collection team which request the second container was intended to accompany.

07 • Technician: Our receiving lead will review the specimen label and paperwork before resolving this accessioning discrepancy.

08 • Clinic coordinator: Should I describe both as duplicates in the clinic note while we check?

09 • Technician: Not yet. Duplicate would be a conclusion about their relationship, which remains unresolved.

10 • Clinic coordinator: I'll describe a second container with an unclear relationship and get the collection team's account.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 01 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Accessioning and unambiguous identifiers

The receiving log lists accession A314; a clinic caller asks about A341. No identity match has been established. Technician Noor can check the accession log. The caller must identify the requested case through the laboratory's privacy and verification arrangements.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic caller: your note says A341; check the actual request when asked.

Second round: Say both cases came from the same clinic and ask whether that is enough to match them.

Scenario 2 | A shared surname on two requests

Two fictional requests have the same surname but different accession references. A caller uses the surname only. The technician can request the full required verification; neither case status has been disclosed.

Role A: Technician: explain why the surname does not identify one case.

Role B: Caller: you have the request reference but initially omit it.

Possible opening: I'm checking the Morgan sample from this morning.

Success checks

- Ask for the required reference and identity verification.
- Avoid choosing a case by arrival order.
- Read back the established reference before discussing its status.

Add a complication: The caller assumes the earlier arrival must be the intended case.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 02 / READ AND NOTICE

Specimen suitability and recollection language

Explain suitability wording without interpreting health status or teaching collection.

SITUATION

The laboratory lead has documented insufficient quantity for test request Q6 and requested recollection through the clinic. No collection method, urgency, or cause is supplied. The clinic thinks insufficient quantity means a low patient result.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

hemolysis - Breakdown of red blood cells that can affect some laboratory measurements, requiring method-specific assessment.

insufficient quantity - A specimen amount that does not meet the laboratory's requirements for the requested testing.

recollection - Obtaining another specimen when the laboratory or care team determines one is needed.

acceptance criteria - The laboratory's defined requirements for determining whether a specimen is suitable for specified testing.

Useful word combinations

review specimen suitability: assess whether the specimen meets the relevant requirements

state the rejection reason: report the documented reason testing cannot proceed on that specimen

request recollection clarification: ask which specimen or request a recollection decision concerns

avoid assigning collection blame: describe a specimen issue without inventing who caused it

LESSON 02 / LANGUAGE AND PRACTICE

Distinguish a specimen problem from a patient result

3. Notice the language

Insufficient quantity concerns available sample, not a measured patient level. State the documented decision and its request reference. Do not generalize one specimen issue to every requested test.

___ describes the specimen, not a patient result.

The documented decision for request ___ is ___.

The care team needs to confirm ___.

Compare this wording: The patient's result is too low to report.

Clearer: The specimen quantity is insufficient for Q6; no reportable result is available from that request.

This prevents a suitability problem from sounding like a clinical finding.

4. Check the language

1. Clinic: Does insufficient quantity mean the patient's level is low?

Technician: No, it describes the available ____.

- A. clinical diagnosis
- B. measured concentration
- C. reference interval
- D. specimen amount

2. Clinic: Was a second sample already obtained?

Technician: Recollection was ____, but completion has not been confirmed.

- A. cancelled
- B. completed
- C. requested
- D. reported as received

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 02 / 05 • CONVERSATIONS / 1 OF 3

Too little specimen, not a low result

Fictional Q6 call using the lesson case.

01 • Clinic coordinator: The note says insufficient quantity. Does that mean the patient's measured level was low?

02 • Technician: No. It describes the amount of specimen available for Q6, not a measured patient result.

03 • Clinic coordinator: So there isn't a number I should pass to the clinician from this request?

04 • Technician: Correct. Our acceptance criteria aren't met. The lead requested recollection for Q6; no result is reportable.

05 • Clinic coordinator: Does the request tell us when or how to collect another specimen?

06 • Technician: This call provides no collection instructions. Your care team needs to arrange that under its procedures.

07 • Clinic coordinator: I'll pass the documented request to the clinician rather than tell the patient a result was low.

08 • Technician: Please include Q6 so the team knows which request the recollection message concerns.

09 • Clinic coordinator: Can we say the original collector caused the insufficient amount to avoid confusion?

10 • Technician: The cause isn't established. Report the documented quantity issue without assigning blame for it.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 2 OF 3

Hemolysis is not a diagnosis

Fictional enquiry about a specimen comment; method-specific assessment remains with the laboratory lead.

01 • Clinic nurse: The specimen comment mentions hemolysis. Can you tell me what the patient has?

02 • Technician: The comment describes a specimen condition. It isn't, by itself, a diagnosis of the patient.

03 • Clinic nurse: Does it mean every test from that collection must be considered unusable?

04 • Technician: That depends on the laboratory's assessment for the relevant tests. I can't generalize from this comment.

05 • Clinic nurse: Could you ask the laboratory lead which requested tests the recorded decision covers?

06 • Technician: Yes. I'll ask for the scope of the decision under the accession reference.

07 • Clinic nurse: Please don't send a general recollection message if only one request is affected.

08 • Technician: I'll relay the lead's documented decision, including its scope, rather than invent broader instructions.

09 • Clinic nurse: We need that distinction before we explain the situation to the ordering clinician.

10 • Technician: I'll mark your question as decision scope and ask the lead to clarify it with your team.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 05 • CONVERSATIONS / 3 OF 3

Recollection requested, arrival unknown

Fictional specimen-status call; clinic says a new collection was planned, not confirmed.

01 • Technician: We're following up on the recollection request for S12. Has a new specimen been obtained?

02 • Clinic coordinator: It was planned for this morning, but I haven't confirmed whether the appointment took place.

03 • Technician: Then collection and laboratory receipt both remain unconfirmed. I'll avoid marking a new specimen received.

04 • Clinic coordinator: Could you tell the clinician that the plan is in place, even though completion is unknown?

05 • Technician: Yes, provided I label it planned. That wording doesn't imply the collection actually happened.

06 • Clinic coordinator: I'll check the attendance and collection records with our team and return the verified status.

07 • Technician: Please use S12 and distinguish the original specimen from any replacement in your reply.

08 • Clinic coordinator: Should I say the original request is resolved because we've arranged the appointment?

09 • Technician: No. The new appointment doesn't establish a suitable replacement or completed testing.

10 • Clinic coordinator: Understood. I'll report the confirmed stage after checking, rather than close the recollection enquiry early.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 02 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Specimen suitability and recollection language

The laboratory lead has documented insufficient quantity for test request Q6 and requested recollection through the clinic. No collection method, urgency, or cause is supplied. The clinic thinks insufficient quantity means a low patient result.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic coordinator: you think insufficient quantity refers to a low measured result.

Second round: Ask whether the collector definitely caused the problem; the cause is not established.

Scenario 2 | One rejected request, another still pending

A laboratory lead rejected specimen use for request J1 under documented acceptance criteria. Request J2 has no decision yet. The clinic assumes both were rejected. No technical cause or collection directions are supplied.

Role A: Technician: limit the rejection statement to J1.

Role B: Clinic nurse: ask whether J2 also needs recollection.

Possible opening: Should we tell the patient that both tests need another sample?

Success checks

- State J1's documented rejection.
- Keep J2's decision pending.
- Ask the lead to clarify J2 without independently ordering recollection.

Add a complication: Both requests share a collection visit, but their suitability decisions differ in status.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 03 / READ AND NOTICE

Quality-control status and reportable results

Explain why a completed run is not necessarily a released report.

SITUATION

An analytical run is complete, but its quality-control review is unresolved. The laboratory lead has not authorized result release. A clinic wants results described as final because the instrument finished. No values or troubleshooting instructions are supplied.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

quality control - Checks used to monitor whether a laboratory testing process is performing within its established requirements.

control material - Material with assigned or expected characteristics used to monitor a test system's performance.

analytical run - A defined group or sequence of laboratory testing performed under specified conditions.

release authorization - The required approval allowing laboratory results to be made available for reporting.

Useful word combinations

review control performance: assess the recorded quality-control information

hold results for review: keep results from release while a required issue is unresolved

confirm run status: establish whether a testing run has met its review requirements

separate completion from release: distinguish finished instrument work from permission to report

LESSON 03 / LANGUAGE AND PRACTICE

Resist an inaccurate final-status label

3. Notice the language

Describe the stage without teaching control interpretation or corrective testing. Unresolved review does not establish that every patient result is wrong. Keep the release decision with the laboratory's qualified reviewer.

The run is complete; ___ remains unresolved.

Completion does not establish ___.

I can confirm ___, but cannot report ___ as final.

Compare this wording: The analyzer finished, so the report is final.

Clearer: The run finished, but quality review and release authorization remain outstanding.

Instrument completion and reporting permission answer different questions.

4. Check the language

1. Clinic: The run finished. Are results released?

Technician: Not yet; the quality-control review remains ____.

- A. archived
- B. completed
- C. unresolved
- D. waived

2. Lead: What checks the test system rather than representing this patient's specimen?

Technician: The ____.

- A. patient aliquot
- B. control material
- C. recollection specimen
- D. referral specimen

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 03 / 05 • CONVERSATIONS / 1 OF 3

Finished on the instrument, awaiting review

Fictional clinic enquiry using the quality-control case.

01 • Clinic caller: The analyzer has finished, so can you send the final results to our team?

02 • Technician: The analytical run is complete, but quality-control review is unresolved and release hasn't been authorized.

03 • Clinic caller: Our clinician is waiting. Could you at least call the results final on the status screen?

04 • Technician: Final would be inaccurate. I can report completed testing with review and release still outstanding.

05 • Clinic caller: Does that mean all the numbers must be wrong, or that nobody has looked?

06 • Technician: Neither conclusion follows from this status. The laboratory lead is reviewing the issue.

07 • Clinic caller: When will that review finish? We need a definite time for the patient.

08 • Technician: I don't have a confirmed release time. I'll pass the timing concern to the lead.

09 • Clinic caller: Please make the difference explicit in your update, so reception doesn't promise a finished report.

10 • Technician: I will: run complete, quality review unresolved, and no release authorization confirmed.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 2 OF 3

Control material is not a patient result

Fictional new colleague misreads the source of an entry; no numerical results.

01 • Technician: The entry you quoted is labeled control material, not a patient specimen result.

02 • Colleague: I saw it beside the run record and assumed it belonged to the patient accession.

03 • Technician: The control entry monitors the testing process. Its position doesn't make it a patient result.

04 • Colleague: Then I shouldn't copy that entry into a patient-status message or discuss its clinical meaning.

05 • Technician: Correct. First establish the source and record type; keep the run review with the lead.

06 • Colleague: Could you show me the label that distinguishes the two records, without interpreting the control?

07 • Technician: The source field says control material here; the patient record uses its accession reference.

08 • Colleague: I see the distinction now. My question concerns record identity, not whether performance is acceptable.

09 • Technician: I'll ask the lead to review any performance question. We've resolved which entry you were reading.

10 • Colleague: Thanks. I'll withdraw the unsent patient-status wording and check the correct accession record instead.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 05 • CONVERSATIONS / 3 OF 3

A run includes more than one request

Fictional result-status query; one reviewed request does not establish the whole run's status.

01 • Clinic nurse: We received a final report for one request. Does that mean everything in the run is released?

02 • Technician: Not necessarily. A final report identifies its own request; it doesn't confirm every other request's status.

03 • Clinic nurse: Could you check the two accession references separately rather than answer about the instrument generally?

04 • Technician: Yes. The first shows released; the second remains under review, with no release time confirmed.

05 • Clinic nurse: I had assumed the same instrument meant the same reporting stage for both.

06 • Technician: The records show otherwise here. Shared testing equipment doesn't make their statuses interchangeable.

07 • Clinic nurse: Please put the second reference first in the update, because that's the one our clinician needs clarified.

08 • Technician: I'll report its review status explicitly and keep the already released request separate.

09 • Clinic nurse: Should I say there's been a new failure, given that one report is delayed?

10 • Technician: No cause is supplied. A different release stage doesn't by itself establish a new failure.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 03 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Quality-control status and reportable results

An analytical run is complete, but its quality-control review is unresolved. The laboratory lead has not authorized result release. A clinic wants results described as final because the instrument finished. No values or troubleshooting instructions are supplied.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic caller: you need a status but initially equate a finished run with final results.

Second round: Ask whether the quality review proves the patient results are inaccurate.

Scenario 2 | Review assigned but not completed

The laboratory lead has assigned quality review to reviewer Ana. Ana has acknowledged the task but has not reported a decision. A colleague wants to mark the run released.

Role A: Technician: distinguish review ownership from release permission.

Role B: Colleague: you believe Ana's acknowledgment finishes the review.

Possible opening: Ana accepted it, so can I mark the results released now?

Success checks

- Specify that Ana accepted the review task.
- Keep the review decision pending.
- Avoid interpreting control values or authorizing release.

Add a complication: The colleague quotes the word accepted without naming what was accepted.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 04 / READ AND NOTICE

Reagents, instruments, and service coordination

Coordinate equipment status without giving technical operating instructions.

SITUATION

Analyzer H2 is marked unavailable. A service visit ended at 11:00, but the laboratory lead has not confirmed return to service. Technician Noor needs a status from the lead before updating the clinic. No repair steps, settings, or patient results are supplied.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

reagent - A substance used in a laboratory test to produce or measure a reaction.

lot verification - The laboratory's checks establishing whether a particular material lot meets requirements for use.

maintenance record - Documentation of service, checks, and related actions for laboratory equipment.

downtime - A period when a laboratory instrument or service is unavailable for its intended work.

Useful word combinations

identify the reagent lot: specify the exact material batch being discussed

check verification status: establish whether required material review is complete

log instrument downtime: record the period and scope of unavailability

coordinate a service enquiry: give equipment support the facts needed for review

LESSON 04 / LANGUAGE AND PRACTICE

Ask what a service completion actually establishes

3. Notice the language

A service engineer's completed visit and the laboratory's permission to resume testing are separate. State the instrument reference, observed status, and exact question. Do not guess settings, restart methods, or cause.

The service visit ended at ____; ____ remains unconfirmed.

Could you confirm whether ____ is available for ____?

Who owns the next update on ____?

Compare this wording: The engineer left, so H2 is ready.

Clearer: The visit ended; the laboratory's return-to-service confirmation is still needed.

The revised wording does not turn a departure time into operational readiness.

4. Check the language

1. Clinic: The service visit ended. Is testing available again?

Technician: Return to service is still ____.

- A. unconfirmed
- B. completed
- C. recorded as approved
- D. scheduled yesterday

2. Lead: Two reagent batches arrived. What identifies which batch needs verification?

Technician: Its ____.

- A. lot number
- B. specimen accession
- C. appointment reference
- D. instrument name

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 04 / 05 • CONVERSATIONS / 1 OF 3

The engineer left at eleven

Fictional service-status call using the H2 case.

01 • Technician: The maintenance record shows the visit ended at eleven. Is H2 confirmed back in service?

02 • Laboratory lead: Not yet. The completed visit doesn't establish the laboratory's readiness decision for testing.

03 • Technician: The clinic asked for a status. Should I say the instrument is still unavailable?

04 • Laboratory lead: Yes, that is the current status. Do not describe the service visit as permission to resume.

05 • Technician: They also asked whether an illuminated screen means the problem has been resolved.

06 • Laboratory lead: A visible screen doesn't establish our required readiness checks. Keep the update to confirmed status.

07 • Technician: Who will provide the next update so the clinic isn't relying on another visual guess?

08 • Laboratory lead: I will. I haven't confirmed a time, so please don't invent one for them.

09 • Technician: I'll say H2 remains unavailable, the service visit is complete, and your review is outstanding.

10 • Laboratory lead: That is accurate. Send equipment-operation questions to me rather than attempting to explain the repair.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 2 OF 3

Received does not mean verified

Fictional reagent shipment; lot V8 received but lot verification is incomplete.

01 • Technician: The new reagent lot V8 is here. Has its verification status been confirmed?

02 • Stock coordinator: Receipt is logged, but the required lot verification is still pending with the laboratory lead.

03 • Technician: Then I'll describe it as received, not available for testing on the strength of delivery alone.

04 • Stock coordinator: Please do. The purchase order's completed status refers to the delivery, not the technical review.

05 • Technician: Could you identify the lot in the update? There is another batch with similar packaging.

06 • Stock coordinator: Yes, I'll use V8 explicitly and avoid referring to it as just the new box.

07 • Technician: I also need to know whether a different lot is available; can you confirm that?

08 • Stock coordinator: I can't confirm another lot's status from this entry. The lead needs to check that separately.

09 • Technician: I'll ask the lead about availability without treating the two batches as interchangeable.

10 • Stock coordinator: That keeps the stock enquiry precise and avoids turning a receipt record into a use decision.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 05 • CONVERSATIONS / 3 OF 3

A service reference without patient details

Fictional support call about a display message; no patient data or repair steps are shared.

01 • Technician: I'm following up on service reference H19 for analyzer H2. Has your engineer accepted the enquiry?

02 • Service coordinator: Yes, an engineer is assigned, but we haven't confirmed a visit or resolution time.

03 • Technician: Our clinic team needs an update. Can I quote an assigned engineer rather than a scheduled visit?

04 • Service coordinator: Exactly. Assignment confirms ownership, not travel arrangements or completion of the repair.

05 • Technician: Do you need patient results in this call, or only the equipment reference and reported display message?

06 • Service coordinator: Only the approved equipment information for this enquiry. Don't send patient details with the service reference.

07 • Technician: I'll use the designated support channel and the description already recorded by our lead.

08 • Service coordinator: Please keep the message wording exact rather than paraphrase it as a confirmed technical diagnosis.

09 • Technician: Understood. Engineer assigned, visit unconfirmed; the cause remains with the service review.

10 • Service coordinator: I'll update the laboratory lead through the existing ticket when there is a confirmed scheduling change.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 04 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Reagents, instruments, and service coordination

Analyzer H2 is marked unavailable. A service visit ended at 11:00, but the laboratory lead has not confirmed return to service. Technician Noor needs a status from the lead before updating the clinic. No repair steps, settings, or patient results are supplied.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Laboratory lead: you have not confirmed return to service; ask what the technician needs for the clinic update.

Second round: Ask whether the instrument's illuminated screen proves it is available for testing.

Scenario 2 | A maintenance date from the wrong instrument

A colleague quotes a completed maintenance record for H1 while answering an enquiry about H2. H2 remains unavailable. The laboratory lead can review both references.

Role A: Technician: identify the scope mismatch and request H2's actual status.

Role B: Colleague: you saw a similar instrument name and assumed the record applied.

Possible opening: That maintenance entry names H1; the clinic is asking about H2.

Success checks

- Repeat both instrument references clearly.
- Avoid transferring one instrument's completion status to another.
- Request the lead's H2 status without giving operating advice.

Add a complication: Both instruments are in the same room, which the colleague treats as confirmation.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 05 / READ AND NOTICE

Turnaround times and realistic updates

Explain laboratory timing without inventing a guaranteed deadline.

SITUATION

The service's quoted two-hour turnaround is measured from laboratory receipt to reporting. Specimen T5 was collected at 08:00 and received at 09:00. A backlog affects estimates; no report time is confirmed. The clinic assumes collection started the two-hour clock.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

turnaround time - The interval between defined starting and ending events in a laboratory service, such as receipt and reporting.

STAT priority - A designation for urgent testing under the laboratory's established priority and response arrangements.

backlog - Laboratory work waiting to be completed beyond the work currently being processed.

add-on request - A request to perform additional testing on an existing specimen, subject to order and suitability requirements.

Useful word combinations

define the turnaround clock: state which events start and end the quoted interval

update the expected reporting time: revise a timing estimate using current information

clarify the stated priority: check the requested urgency against the recorded order

assess an add-on request: obtain review of whether extra testing can be accepted

LESSON 05 / LANGUAGE AND PRACTICE

Name the starting point before discussing the estimate

3. Notice the language

Turnaround intervals need defined start and end events. Correcting the starting point does not eliminate delays. STAT is a priority designation, not a promise that a particular result already exists.

Our quoted interval runs from ___ to ___.

___ is the reference point, not ___.

The current estimate is affected by ___; ___ remains unconfirmed.

Compare this wording: Two hours after collection is definitely the reporting deadline.

Clearer: This interval starts at receipt; the current backlog means a report time is still unconfirmed.

The timing explanation identifies both the clock and the uncertainty.

4. Check the language

1. Clinic: When does this service's turnaround clock start?

Technician: It starts at laboratory ___, not collection.

- A. dispatch
- B. release
- C. recollection
- D. receipt

2. Clinic: Can an add-on be guaranteed because the specimen is already there?

Technician: No. The additional request still needs ___.

- A. suitability review
- B. collection-time correction
- C. automatic duplication
- D. a completed report

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 05 / 05 • CONVERSATIONS / 1 OF 3

Which clock starts at nine?

Fictional T5 timing call using the lesson case.

01 • Clinic coordinator: T5 was collected at eight. Doesn't the two-hour turnaround time mean reporting by ten?

02 • Technician: This service measures turnaround from laboratory receipt to reporting. Receipt was at nine, not eight.

03 • Clinic coordinator: Then should I promise the clinician a report at eleven instead of ten?

04 • Technician: No. The backlog affects the estimate, so changing the starting point doesn't guarantee eleven.

05 • Clinic coordinator: Could the section lead provide a current estimate rather than just repeat the usual interval?

06 • Technician: Yes, I'll ask for the latest timing information with T5's reference and receipt time.

07 • Clinic coordinator: We need to distinguish an overdue prediction from a specimen that hasn't arrived.

08 • Technician: Receipt is confirmed. The open question is reporting time, not whether the laboratory has T5.

09 • Clinic coordinator: I'll explain that to the clinician and avoid describing collection time as your receipt time.

10 • Technician: I'll pass on the timing query and keep any new estimate clearly labeled.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 2 OF 3

The add-on is a new request

Fictional clinic requests extra testing on an existing specimen; no technical suitability decision.

01 • Clinic nurse: The specimen is already with you. Can we add the extra test without another discussion?

02 • Technician: An add-on request still needs the appropriate order and specimen-suitability review.

03 • Clinic nurse: I thought an existing accession meant every additional test could use the same material.

04 • Technician: It identifies the case, but doesn't establish that the specimen meets every new test's requirements.

05 • Clinic nurse: Could you send the add-on request to the section lead for that review?

06 • Technician: Yes. I'll keep it linked to the original accession while distinguishing the additional requested test.

07 • Clinic nurse: Please tell us whether it was accepted rather than assuming the original testing approval covers it.

08 • Technician: That's the status I'll request. Acceptance and reporting time will need separate confirmation.

09 • Clinic nurse: We haven't told the patient to return for another collection; that decision isn't made.

10 • Technician: I'll preserve that uncertainty and report the lead's decision, without adding collection instructions myself.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 05 • CONVERSATIONS / 3 OF 3

A priority label needs confirmation

Fictional clinician enquiry says urgent; recorded order priority has not been checked.

01 • Technician: Your message says STAT priority, but I need to clarify the recorded order priority.

02 • Clinic coordinator: The clinician asked for an urgent update. I may have used STAT to mean that.

03 • Technician: An urgent request for information isn't automatically a changed testing priority. Let's distinguish them.

04 • Clinic coordinator: We need the current status first, and the clinician can clarify any priority request.

05 • Technician: I'll route that question to the laboratory lead with the order reference.

06 • Clinic coordinator: Please don't change the clinical request based only on my shorthand in this telephone message.

07 • Technician: I won't. I'll describe your message as an urgent status enquiry, with order priority to verify.

08 • Clinic coordinator: Does that wording mean you are deciding it isn't clinically urgent?

09 • Technician: No. I'm not making that assessment; I'm identifying the information that needs the responsible team's clarification.

10 • Clinic coordinator: Thank you. I'll ask the clinician to confirm the intended priority through the proper order route.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 05 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Turnaround times and realistic updates

The service's quoted two-hour turnaround is measured from laboratory receipt to reporting. Specimen T5 was collected at 08:00 and received at 09:00. A backlog affects estimates; no report time is confirmed. The clinic assumes collection started the two-hour clock.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic coordinator: count two hours from the 08:00 collection and ask why no report is available.

Second round: Ask whether adding STAT to your telephone message independently changes the recorded order priority.

Scenario 2 | A revised estimate after a backlog update

At 10:00 the section lead estimated reporting at noon. At 11:00 the lead says the estimate is no longer supported because the backlog changed. No replacement time is confirmed; a status update is agreed for 12:30.

Role A: Technician: correct the estimate without inventing a new deadline.

Role B: Clinic coordinator: you planned a noon results discussion.

Possible opening: We scheduled around noon. Has the report time changed?

Success checks

- Identify noon as the superseded estimate.
- Keep the new report time unknown.
- State that 12:30 is a status-update commitment.

Add a complication: The coordinator treats 12:30 as the replacement report deadline.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 06 / READ AND NOTICE

Preliminary, final, and amended reports

Repair an inaccurate version statement without interpreting results.

SITUATION

The clinic received a preliminary report at 10:00 and a final report at 14:00. A technician called the earlier version final by mistake. The final report supersedes the preliminary version for this request. No findings or clinical conclusions are supplied.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

preliminary report - A report issued before all relevant testing or review is complete, with its stated limitations.

final report - A report issued at the completed reporting stage for the specified request, subject to any later correction.

amended report - A report revised after issue, with the change recorded through the laboratory's correction process.

reference interval - A comparison range supplied with some test results, derived for a defined reference population and method.

Useful word combinations

identify the report version: state which issued document is being discussed

explain preliminary status: distinguish an interim report from a completed reporting stage

notify a report amendment: tell the receiving team that an issued report has changed

refer interpretation questions: direct clinical meaning questions to the responsible clinician

LESSON 06 / LANGUAGE AND PRACTICE

Correct the report label and verify the version

3. Notice the language

A final label describes a reporting stage, not an assurance that a patient is healthy or that no correction could ever follow. Use the reference, issue time, and version together.

I incorrectly called ___ final; it was ___.

The version issued at ___ supersedes ___.

Please confirm which report your team is viewing.

Compare this wording: Final means the patient is fine.

Clearer: Final describes the report's stage; the clinician needs to explain its meaning for the patient.

Reporting status and clinical interpretation must remain distinct.

4. Check the language

1. Clinic: Which word describes an issued report changed afterward through the correction process?

Technician: ____.

- A. Amended
- B. Unordered
- C. Preliminary
- D. Unreceived

2. Clinic: Does final mean the clinician has explained it to the patient?

Technician: No. It describes the report's ____, not the completed discussion.

- A. patient agreement
- B. reporting stage
- C. clinical reassurance
- D. appointment purpose

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 06 / 05 • CONVERSATIONS / 1 OF 3

The two o'clock version

Fictional correction call using the lesson case.

01 • Technician: I need to correct my wording. The ten o'clock preliminary report wasn't final.

02 • Clinic nurse: We printed that version for the clinician. Is there now a different report?

03 • Technician: Yes, the final report was issued at two and supersedes the preliminary version for this request.

04 • Clinic nurse: Please give the issue time again so I can identify the correct document.

05 • Technician: Two o'clock is the final version. I can clarify the version, not interpret its findings.

06 • Clinic nurse: I have it open now. The heading and issue time both match your description.

07 • Technician: Could you alert the clinician that the earlier printout was preliminary and has been superseded?

08 • Clinic nurse: I will. Should I say the newer report proves the patient has no problem?

09 • Technician: No. That is a clinical conclusion the report's final status does not establish.

10 • Clinic nurse: Understood. I'll identify the current version and ask the clinician to interpret it with the patient.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 2 OF 3

A reference interval isn't the whole answer

Fictional patient-support call; no values or diagnoses are supplied.

01 • Clinic nurse: A caller asks whether being within the reference interval means every concern is ruled out.

02 • Technician: The interval is a comparison range for that result, not a conclusion about every clinical concern.

03 • Clinic nurse: Could you confirm the report's wording while leaving the patient's interpretation question with the clinician?

04 • Technician: Yes. I can identify the interval printed on the report and the version being discussed.

05 • Clinic nurse: The caller also compares it with a report from another laboratory using a different range.

06 • Technician: Different methods and reference populations may use different intervals; the clinician should review the actual reports.

07 • Clinic nurse: Please don't tell them one laboratory must be wrong simply because the ranges differ.

08 • Technician: I won't. A difference in printed ranges doesn't establish an error by itself.

09 • Clinic nurse: I'll arrange the clinician's explanation and make sure both reports are available for that discussion.

10 • Technician: That gives the clinician the sources and preserves the distinction between comparison information and clinical interpretation.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 05 • CONVERSATIONS / 3 OF 3

An amended report needs a notified recipient

Fictional laboratory has issued an amended report; receiving clinic has not acknowledged the notification.

01 • Technician: I'm contacting you about an amended report for request R22 issued this afternoon.

02 • Clinic coordinator: We already have a final report from yesterday. Is this just another copy of that one?

03 • Technician: No. Amended means an issued report has been revised through the laboratory's correction process.

04 • Clinic coordinator: Can you identify the version without discussing patient information on this unverified line?

05 • Technician: Yes. We'll use the established verified contact route for the report details and notification.

06 • Clinic coordinator: I can arrange that route now. Should I mark the clinician notified before we connect?

07 • Technician: Not yet. Arranging a connection doesn't establish that the responsible recipient received the amendment.

08 • Clinic coordinator: I'll connect the designated recipient and keep notification pending until the handoff is acknowledged.

09 • Technician: Thank you. The amendment's clinical meaning belongs with the clinician, but its changed version must be clear.

10 • Clinic coordinator: I'll identify the message as an amendment notification, not a routine duplicate report.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 06 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Preliminary, final, and amended reports

The clinic received a preliminary report at 10:00 and a final report at 14:00. A technician called the earlier version final by mistake. The final report supersedes the preliminary version for this request. No findings or clinical conclusions are supplied.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic nurse: you have the ten o'clock printout and need to identify the current version.

Second round: Ask whether a value inside a reference interval settles the whole clinical question.

Scenario 2 | A portal upload mistaken for clinical review

A final report is visible in the portal. The ordering clinician has not confirmed reviewing it or contacting the patient. A caller assumes visibility means the clinical discussion is complete.

Role A: Technician: explain report availability without implying clinical review.

Role B: Clinic coordinator: ask which follow-up task still needs a named owner.

Possible opening: It's in the portal, so can we close the patient-explanation task?

Success checks

- Separate report availability, viewing, and explanation.
- Do not interpret the findings.
- Request confirmation of the clinical follow-up owner.

Add a complication: The caller treats a portal view count as evidence of understanding.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 07 / READ AND NOTICE

Critical notifications and acknowledged handoffs

Communicate a fictional result accurately and confirm its receipt by the stated recipient.

SITUATION

Fictional laboratory Lumen designates C7's potassium result of 6.4 mmol/L critical under its own criteria; mmol/L means millimoles per liter. Case identity and Dr Shah's receiving authority are already verified. The report was issued at 14:10; this call occurs at 14:12. Dr Shah's contact is extension 214. Dr Shah initially repeats 6.1. Correct the value, obtain the complete read-back, and confirm receipt. No diagnosis, treatment, or universal threshold is supplied.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

critical result - A result meeting the laboratory's established criteria for prompt notification through its designated clinical pathway.

read-back - Repeating communicated details so the sender can check that they were received accurately.

notification log - The laboratory record of result-notification attempts, recipients, acknowledgments, and times.

escalation pathway - The designated sequence of contacts and actions used when the required response or recipient is not reached.

Useful word combinations

verify the receiving clinician: establish that the intended qualified recipient is on the line

obtain a read-back: request repetition of the critical communicated details

record acknowledgment: document that the intended recipient confirmed receipt

escalate an unanswered notification: use the designated pathway when the required contact is not achieved

LESSON 07 / LANGUAGE AND PRACTICE

Complete the read-back and correct what was misheard

3. Notice the language

The sender states the information, the receiver repeats it, and the sender confirms or corrects it. Analyte means the substance measured. Millimoles per liter (mmol/L) and milligrams per deciliter (mg/dL) are different units; do not interchange or convert them in these exercises. All critical designations are fictional lab-specific facts, not universal thresholds.

For verified case __: [analyte], [value], [units].

You repeated __; the reported value is __. Please read it back.

That read-back matches. Please confirm you accept this notification.

Compare this wording: You heard a number, so I'll mark it done.

Clearer: You repeated six point one; the report says six point four millimoles per liter. Please repeat the complete result.

A corrected number needs a fresh complete read-back, followed by the sender's confirmation.

4. Check the language

1. Technician: C7 potassium is 6.4 mmol/L, meaning millimoles per liter.

Clinician: Six point one?

Technician: __; please repeat the full result.

A. The analyte is sodium

B. The value is six point four

C. The value is six point one

D. The units are milligrams per deciliter

2. Clinician: C7, potassium, six point four millimoles per liter.

Technician: __. Dr Shah, please confirm receipt.

A. That changes the reported result

B. That read-back matches the report

C. That establishes a treatment plan

D. That confirms every laboratory uses this threshold

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 07 / 05 • CONVERSATIONS / 1 OF 3

Six point four, not six point one

All facts fictional: Lumen designates C7 potassium 6.4 mmol/L critical under its own criteria. Identity and Dr Shah's receiving authority are already verified. Report issued 14:10, call at 14:12, contact extension 214. No universal threshold, diagnosis, or treatment is implied.

01 • Technician: Dr Shah, Noor from Lumen laboratory. For C7, the critical potassium result is six point four millimoles per liter.

02 • Dr Shah: C7, potassium, six point one millimoles per liter. Have I heard that correctly?

03 • Technician: It's six point four, not six point one. Please repeat the complete result.

04 • Dr Shah: C7, potassium, six point four millimoles per liter. Is that the value on the report?

05 • Technician: That matches, thank you. The report was issued at two ten this afternoon.

06 • Dr Shah: Is two ten the report's issue time, rather than the specimen collection time?

07 • Technician: Yes, two ten is the issue time. This call is taking place at two twelve.

08 • Dr Shah: Please record Dr Shah as the recipient. I've received C7's result, and my extension is 214.

09 • Technician: I'll record your acknowledgment at two twelve and extension two one four. Is that correct?

10 • Dr Shah: Yes, extension two one four. I have the potassium result and will review the report now.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 2 OF 3

The escalation contact accepts S9

Fictional Lumen case S9: sodium 121 mmol/L is designated critical by this lab. Report issued 11:06. First contact was unanswered at 11:07; immediate local escalation reached verified receiving nurse Elena at 11:08, extension 305. These are case facts, not a required waiting interval. No universal threshold or treatment is implied.

01 • Technician: Elena, Noor at Lumen laboratory. The first line was unanswered, so I'm calling about S9.

02 • Nurse Elena: Elena speaking. I'm covering S9 and can take the result. Please go ahead.

03 • Technician: The critical sodium result is one hundred twenty-one millimoles per liter, issued at eleven oh six.

04 • Nurse Elena: S9, sodium, one hundred twenty-one millimoles per liter. Please check that against the report.

05 • Technician: That read-back matches. Have you received the result and accepted the notification for S9?

06 • Nurse Elena: Yes, I have. Please use my name, Elena, and extension 305 for this contact.

07 • Technician: Elena, extension three zero five. This call is at eleven oh eight; the earlier attempt was unanswered.

08 • Nurse Elena: That's correct. Eleven oh eight is when I received it, not the earlier call time.

09 • Technician: I'll enter this acknowledgment separately in the notification log. The report is available under S9.

10 • Nurse Elena: Thank you. I have the result and units, and I'll bring the report to the care team.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 05 • CONVERSATIONS / 3 OF 3

The unit needs correcting too

Fictional Lumen case G4: glucose 48 mg/dL, meaning milligrams per deciliter, designated critical under this lab's criteria. Identity and receiving nurse Morgan's authority are verified. Report issued 13:40; Morgan's extension is 306. No universal threshold, diagnosis, or treatment is supplied.

01 • Technician: Morgan, Noor from Lumen laboratory. G4's critical glucose result is forty-eight milligrams per deciliter.

02 • Nurse Morgan: G4, glucose, forty-eight millimoles per liter. Is that the complete result you reported?

03 • Technician: The units are milligrams per deciliter, not millimoles per liter. The value is forty-eight.

04 • Nurse Morgan: I misheard the units. Please say the whole result once more so I can check it.

05 • Technician: For G4: glucose, forty-eight milligrams per deciliter. Please read that back to me.

06 • Nurse Morgan: G4, glucose, forty-eight milligrams per deciliter. That's the result I have recorded now.

07 • Technician: That read-back matches the report issued at one forty. Please confirm who is accepting it.

08 • Nurse Morgan: Morgan. I accept the notification, and you can reach me on extension 306.

09 • Technician: Morgan, extension three zero six. I'll record your acknowledgment and the corrected units; is that contact right?

10 • Nurse Morgan: Yes, three zero six. I have G4's glucose result in milligrams per deciliter.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 07 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Critical notifications and acknowledged handoffs

Fictional laboratory Lumen designates C7's potassium result of 6.4 mmol/L critical under its own criteria; mmol/L means millimoles per liter. Case identity and Dr Shah's receiving authority are already verified. The report was issued at 14:10; this call occurs at 14:12. Dr Shah's contact is extension 214. Dr Shah initially repeats 6.1. Correct the value, obtain the complete read-back, and confirm receipt. No diagnosis, treatment, or universal threshold is supplied.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Dr Shah: identity and receiving authority are verified. Initially repeat potassium 6.1 mmol/L for C7; after correction, repeat C7, potassium 6.4 mmol/L and accept the notification.

Second round: Ask whether six point four proves a diagnosis. The technician preserves the reported value and leaves clinical interpretation to the clinician.

Scenario 2 | A decimal lost in the repeat

Fictional Lumen case M2 has magnesium 0.8 mg/dL, meaning milligrams per deciliter, designated critical by this lab. Case identity and Dr Rao's receiving authority are verified. Dr Rao initially repeats eight, omitting zero point. No diagnosis, treatment, or universal threshold is supplied.

Role A: Technician: state M2, magnesium, zero point eight milligrams per deciliter; correct the repeat and verify the final read-back.

Role B: Dr Rao: initially say eight, then repeat the complete corrected result and accept receipt.

Possible opening: For verified case M2, Lumen's critical magnesium result is zero point eight milligrams per deciliter.

Success checks

- Correct eight to zero point eight without converting units.
- Obtain M2, magnesium, zero point eight milligrams per deciliter in one read-back.
- Confirm the accurate repeat and recipient acknowledgment without interpreting the result.

Add a complication: Dr Rao asks whether eight and zero point eight are merely different ways of saying the same value; they are not.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

LESSON 08 / READ AND NOTICE

Referral laboratories and outstanding work

Give a send-out handoff with a clear remaining task and accepted owner.

SITUATION

Send-out X8 appears on Monday's manifest. The courier confirms parcel delivery Tuesday, but the referral laboratory has not confirmed X8's accession acceptance. Noor's shift ends; incoming technician Eli accepts the enquiry and an agreed 15:00 clinic update. No result date is confirmed.

1. Understand the situation

Identify two confirmed facts, one missing detail, and the person who needs your response. Do not fill gaps with invented facts.

2. Find the words

referral laboratory - An external laboratory that performs testing sent from another laboratory or service.

send-out test - A test whose specimen or material is sent to another laboratory for performance.

shipping manifest - A document listing items included in a shipment.

chain of custody - A documented record of possession and transfer used when required for a specimen's particular purpose.

Useful word combinations

reconcile the shipping manifest: compare the shipment list with confirmed received items

confirm referral-laboratory acceptance: check whether the external laboratory accepted the request

handover a send-out enquiry: transfer the unresolved external-testing question to a named colleague

trace a shipment discrepancy: establish which transfer or receipt detail remains unknown

LESSON 08 / LANGUAGE AND PRACTICE

Transfer the item-level question, not just the parcel status

3. Notice the language

A manifest lists intended contents; parcel delivery does not itself confirm item acceptance. Chain-of-custody requirements apply only in their proper context, not automatically to every routine specimen.

The manifest lists ____; the receiving laboratory has confirmed ____.

The outstanding question is ____ for accession ____.

Please confirm you accept both ____ and ____.

Compare this wording: The courier delivered it, so everything is finished.

Clearer: The parcel arrived; X8's acceptance and the clinic update remain outstanding.

This identifies the item-level gap and the communication still owed.

4. Check the language

1. Colleague: Does the manifest prove the external laboratory accepted every listed specimen?

Technician: No. It lists the shipment's ____, not its acceptance decisions.

- A. intended contents
- B. final results
- C. clinical interpretations
- D. completed reviews

2. Outgoing technician: Can you own the acceptance enquiry and the three o'clock update?

Incoming technician: I accept ____ tasks; result timing remains unknown.

- A. either
- B. both
- C. another
- D. neither

05 • Conversations

The next three pages contain original fictional conversations for this lesson. Each numbered line is one speaking turn. Read the setting before assigning roles.

LESSON 08 / 05 • CONVERSATIONS / 1 OF 3

A delivered parcel with acceptance still open

Fictional X8 shift handoff using the lesson case.

01 • Outgoing technician: Send-out test X8 appears on Monday's shipping manifest. Tuesday's parcel delivery is confirmed, but accession acceptance isn't.

02 • Incoming technician: I'll take the enquiry. Does the clinic have a promised result time or only an update time?

03 • Outgoing technician: Only an update at three. There is no confirmed result date from the referral laboratory.

04 • Incoming technician: I accept that clinic update too. I'll ask the referral laboratory specifically about X8's acceptance.

05 • Outgoing technician: Please don't treat the courier signature as proof they registered every item in the parcel.

06 • Incoming technician: I won't. Parcel delivery and item-level acceptance remain separate in the handoff.

07 • Outgoing technician: The clinic also asked whether this means X8 is lost. We haven't established that.

08 • Incoming technician: I'll describe an acceptance enquiry, not a lost specimen, unless the receiving team confirms more.

09 • Outgoing technician: Then you own the external check and three o'clock update, with no promised report date.

10 • Incoming technician: Accepted. I'll record the response and keep unresolved items visible for the next shift.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 2 OF 3

A manifest entry without receipt confirmation

Fictional send-out coordinator compares a shipment list with the external receiving log.

01 • Technician: Our manifest lists S4 and S5, but your acknowledgment names only S4.

02 • Referral coordinator: I can confirm S4's registration. I need the receiving team to check S5 separately.

03 • Technician: Does the acknowledgment intentionally exclude S5, or is that reason still unknown?

04 • Referral coordinator: The reason is unknown. I shouldn't call S5 rejected simply because it isn't mentioned.

05 • Technician: Could you open an item-level enquiry using the manifest reference and the two separate identifiers?

06 • Referral coordinator: Yes. I'll ask the receiving team to compare the physical receipt information and registration records.

07 • Technician: Please keep S4's confirmed status unchanged while you investigate the separate S5 question.

08 • Referral coordinator: Agreed. I won't turn an unresolved item into an assumption about the entire shipment.

09 • Technician: Our clinic update will say S4 confirmed and S5 under receipt enquiry, without predicting a result date.

10 • Referral coordinator: That matches what I can establish now. I'll return the receiving team's finding when available.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 05 • CONVERSATIONS / 3 OF 3

Custody wording has a specific purpose

Fictional laboratory staff clarify documentation terminology without teaching collection or forensic procedures.

01 • Technician: The clinic asked for chain-of-custody documentation. Is that just another name for our routine courier receipt?

02 • Quality coordinator: No. Chain of custody is a specific documented record of possession and transfers where required.

03 • Technician: So I shouldn't rename a delivery signature as a complete chain-of-custody record.

04 • Quality coordinator: Correct. We need to establish the purpose and applicable requirements through the responsible laboratory team.

05 • Technician: Can you review what the clinic is requesting before we describe the documentation as sufficient?

06 • Quality coordinator: Yes. Send the enquiry through the designated record route, with the request purpose stated.

07 • Technician: They haven't told us the purpose yet. I'll obtain that clarification instead of assuming it.

08 • Quality coordinator: Good. Don't promise that a routine send-out meets requirements that haven't been identified.

09 • Technician: I'll explain that the courier receipt confirms its stated delivery event, not every possible documentation requirement.

10 • Quality coordinator: That is the useful distinction. I'll take the documentation review once the intended purpose is clarified.

Notice, then rehearse

Name each speaker's purpose. Identify one useful expression and the next step or unresolved question. Read the roles aloud, then switch roles.

LESSON 08 / 06 • SAY IT

Two scenarios to practice

Use only the supplied facts. Prepare for two minutes, speak for one minute, then respond to a follow-up. Switch roles and repeat. For solo study, speak both roles aloud.

Scenario 1 | Referral laboratories and outstanding work

Send-out X8 appears on Monday's manifest. The courier confirms parcel delivery Tuesday, but the referral laboratory has not confirmed X8's accession acceptance. Noor's shift ends; incoming technician Eli accepts the enquiry and an agreed 15:00 clinic update. No result date is confirmed.

Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Eli: accept the acceptance enquiry and three o'clock update, keeping result timing unknown.

Second round: Ask whether a courier signature counts as referral-laboratory accession acceptance.

Scenario 2 | The receiving laboratory changes its contact person

Referral laboratory contact Ana has moved roles. Send-out Z3 remains pending. Eli has not accepted the query. The receiving laboratory's service coordinator can assign a new owner; a clinic update is due at 16:00.

Role A: Technician: request a named new contact without claiming the test stopped.

Role B: Service coordinator: you can accept the ownership enquiry, but cannot confirm result timing.

Possible opening: Ana no longer handles Z3. Who has accepted the follow-up now?

Success checks

- Separate contact ownership from specimen suitability.
- Keep result timing unknown.
- Preserve the four o'clock update while resolving who owns the external query.

Add a complication: The clinic assumes a staff change means its specimen must be recollected.

Check: facts accurate | meaning clear | tone appropriate | next action or question explicit

Continue scenario 1 with an AI partner.

Answer workshop

Try the tasks first. The model is one acceptable response to scenario 1, not a script to memorize. Assess your spoken version using the four criteria at the end.

01 | Accessioning and unambiguous identifiers

Editing example: A314 and A341 differ. Let's establish the intended accession before discussing status.

The revised wording prevents a reference guess from becoming an identity claim.

Check 1: A - accession number

A: Correct. The accession number links the laboratory case and records.

B: An invoice total is a payment amount.

C: A reagent code identifies a testing material, not the patient case.

D: An appointment date identifies timing, not the registered case.

Check 2: A - transposed

A: Correct. The last two digits have exchanged positions.

B: Omitted would mean missing, not exchanged.

C: Identical would mean exactly the same sequence.

D: Consecutive means following one another in order.

Model response: Before we discuss a specimen status, could we clarify the accession number? Our receiving log lists A314, while your enquiry says A341. Those are different identifiers. Please check the reference on your request through our verification arrangements. I will keep the enquiries separate until we establish which case you mean; no match has been confirmed yet.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

02 | Specimen suitability and recollection language

Editing example: The specimen quantity is insufficient for Q6; no reportable result is available from that request.

This prevents a suitability problem from sounding like a clinical finding.

Check 1: D - specimen amount

A: A clinical diagnosis is not supplied by this specimen-quantity message.

B: No measured patient concentration is available here.

C: A reference interval is a comparison range.

D: Correct. Quantity concerns how much specimen is available.

Check 2: C - requested

A: Cancelled would mean the request was withdrawn.

B: Completed contradicts the unknown status.

C: Correct. A request is not proof a new specimen was obtained.

D: Reported as received would establish a later stage.

Model response: For Q6, insufficient quantity describes the amount of specimen available, not a low result in the patient's blood. The laboratory lead has requested recollection through your clinic. There is no reportable result from this request in this case. Please confirm that the care team has received the recollection message; the collection arrangements belong with them.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

03 | Quality-control status and reportable results

Editing example: The run finished, but quality review and release authorization remain outstanding.

Instrument completion and reporting permission answer different questions.

Check 1: C - unresolved

A: Archived means stored, not awaiting a decision.

B: Completed would say the review has finished.

C: Correct. Unresolved describes the outstanding review.

D: Waived means the requirement was set aside.

Check 2: B - control material

A: A patient aliquot is part of a patient sample.

B: Correct. Control material monitors test-system performance.

C: A recollection specimen is another patient sample.

D: A referral specimen is sent for external testing.

Model response: The analytical run has finished, but the quality-control review remains unresolved and release has not been authorized. I cannot describe these results as final just because the instrument completed its work. The laboratory lead is reviewing the issue. I can report that status to your team, but I do not have a confirmed release time.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

04 | Reagents, instruments, and service coordination

Editing example: The visit ended; the laboratory's return-to-service confirmation is still needed.

The revised wording does not turn a departure time into operational readiness.

Check 1: A - unconfirmed

A: Correct. No laboratory confirmation has been supplied.

B: Completed wrongly presents readiness as established.

C: Recorded as approved contradicts the missing confirmation.

D: Scheduled yesterday describes planning, not present readiness.

Check 2: A - lot number

A: Correct. The lot number identifies the material batch.

B: A specimen accession identifies a laboratory case.

C: An appointment reference identifies a scheduled visit.

D: The instrument name identifies equipment, not a reagent batch.

Model response: Could you confirm H2's return-to-service status? The service visit ended at eleven, but I do not see the laboratory lead's confirmation that testing can resume. I need to update the clinic accurately. Please distinguish the completed service visit from any checks or release decision still outstanding, and tell me who owns the next status update.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

05 | Turnaround times and realistic updates

Editing example: This interval starts at receipt; the current backlog means a report time is still unconfirmed.

The timing explanation identifies both the clock and the uncertainty.

Check 1: D - receipt

A: Dispatch is sending the specimen onward.

B: Release is a later reporting-stage event.

C: Recollection means obtaining another specimen.

D: Correct. Receipt is the stated starting event.

Check 2: A - suitability review

A: Correct. Presence alone doesn't establish suitability for extra testing.

B: Correcting a timestamp would not establish suitability for the additional test.

C: Duplication would repeat work rather than assess the new request.

D: A completed report is an outcome, not the necessary review.

Model response: The quoted two-hour turnaround is measured from laboratory receipt, not collection. T5 was received at nine after collection at eight. That distinction changes the reference point, but the current backlog means I still cannot promise a report at eleven. The section lead can provide the latest estimate; please treat it as an estimate unless confirmed otherwise.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

06 | Preliminary, final, and amended reports

Editing example: Final describes the report's stage; the clinician needs to explain its meaning for the patient.

Reporting status and clinical interpretation must remain distinct.

Check 1: A - Amended

A: Correct. Amended identifies a revised issued report.

B: Unordered describes a missing request, not a correction.

C: Preliminary describes an interim reporting stage.

D: Unreceived concerns whether an item arrived.

Check 2: B - reporting stage

A: Patient agreement concerns a decision, not this report label.

B: Correct. Final identifies a reporting stage.

C: Clinical reassurance would interpret the patient's situation.

D: Appointment purpose describes a visit rather than report status.

Model response: I need to correct my earlier description. The ten o'clock report was preliminary, not final. The final report was issued at two and supersedes that earlier version for this request. Please confirm that your team is viewing the two o'clock report. I can clarify the version and status, but clinical interpretation belongs with the responsible clinician.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

07 | Critical notifications and acknowledged handoffs

Editing example: You repeated six point one; the report says six point four millimoles per liter. Please repeat the complete result.

A corrected number needs a fresh complete read-back, followed by the sender's confirmation.

Check 1: B - The value is six point four

A: The stated analyte is potassium, not sodium.

B: Correct. This restores the stated value and requests confirmation.

C: Six point one repeats the misheard value.

D: These units differ from the supplied millimoles per liter.

Check 2: B - That read-back matches the report

A: The accurate read-back repeats rather than changes the report.

B: Correct. The sender verifies the receiver's complete repeat.

C: Reading back a value does not establish treatment.

D: The critical designation is specific to this fictional laboratory.

Model response: Dr Shah, Noor from Lumen laboratory. For C7, the critical potassium result is six point four millimoles per liter. You repeated six point one; it is six point four. Please read back the complete result. That matches, thank you. The report was issued at two ten; I'll record your receipt at two twelve and contact extension 214.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

08 | Referral laboratories and outstanding work

Editing example: The parcel arrived; X8's acceptance and the clinic update remain outstanding.

This identifies the item-level gap and the communication still owed.

Check 1: A - intended contents

A: Correct. A manifest identifies listed shipment contents.

B: Final results are produced after testing and review.

C: Clinical interpretations concern meaning for patient care.

D: Completed reviews are decisions, not shipment listings.

Check 2: B - both

A: Either leaves which task is accepted unclear.

B: Correct. Both explicitly covers the two named tasks.

C: Another refers to a different unspecified task.

D: Neither would reject the two responsibilities.

Model response: Eli, X8 is listed on Monday's manifest, and parcel delivery was confirmed Tuesday. We still lack the referral laboratory's accession acceptance for X8, so a result date is not established. Please take the acceptance enquiry and the clinic update agreed for three. Confirm that you have both tasks; parcel delivery alone does not close this send-out follow-up.

Case check: underline the confirmed information and circle a limitation, question, or next step in the response.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and switches roles and repeats the exchange. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

U.S. Bureau of Labor Statistics: Clinical Laboratory Technologists and Technicians

<https://www.bls.gov/ooh/healthcare/clinical-laboratory-technologists-and-technicians.htm>

Centers for Disease Control and Prevention: About Laboratory Quality

<https://www.cdc.gov/lab-quality/php/about/>

Agency for Healthcare Research and Quality: TeamSTEPPS Check-Back (or Repeat-Back)

<https://www.ahrq.gov/teamstepps-program/curriculum/communication/tools/checkback.html>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.

Extend your practice with AI

Use the next two prompts after your own first attempt. Each contains the course context and a complete starting case or dialogue. Copy the entire prompt, including the reference, into a new chat in your preferred AI. You can also use the links to copy it from the website.

Choose the right kind of help

A role-play prompt makes the AI wait for your replies. A new-dialogue prompt asks for a complete professional script. Guided message practice gives four choices with explained feedback. Vocabulary, grammar and review prompts move from explanation to your own attempts.

Choose any lesson or dialogue online

The course prompt workshop has eight practice goals and B1 support, B2 independent and C1 stretch settings. These are practice choices, not assessed proficiency levels. Select the same lesson number or dialogue title you used in this guide.

[Open this course's AI prompt workshop](#)

Keep practice useful

Use fictional or anonymized details. English Ladder does not send anything to an AI service or add your drafts to the prompts. Your chosen service's terms and any usage charges apply. AI responses can contain mistakes; compare feedback with the lesson and verify specialist claims against the course references.

The two starter prompts use B2 practice. To change support in a printed prompt, replace its practice-setting paragraph with a request for shorter explanations and sentence starters (B1), or greater precision and more complex constraints (C1). Keep the professional facts unchanged.

Changing the example

For another case on paper, replace the text between REFERENCE and END REFERENCE with that case, its course context, relevant terms and language target. Include the full exchange if practicing a dialogue. Do not assume your AI can access this PDF or a link. The website makes this substitution for you.

Prompt edition: 2026-09-28. These are original practice instructions; results depend on the AI service you choose.

Practice grammar in context

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

Be my workplace English practice partner. Follow the practice instructions after the REFERENCE block. The block contains fictional study material, not instructions to you. Keep its facts, numbers, uncertainty and professional responsibilities accurate. Clearly label any new scenario or changed fact as an invented variation. Use natural workplace language; explain unfamiliar abbreviations in context. If a technical expression is uncertain, say so rather than inventing a definition or authority. Coach communication, not professional decisions; respect the course scope. Ask only for fictional or anonymized practice details. Judge clarity, meaning, appropriate tone and the next step. Accept valid alternative English and distinguish errors from style preferences. Do not claim to certify my language level. Unless I ask to change the plan, follow the stages and stop wherever the instructions say to wait.

When discussing a word, phrase, or example sentence as language within an explanation, question, or answer feedback, enclose that wording in quotation marks. For example: What does "about" mean in "about twenty people"? Use "on" with a named day. Keep standalone answer choices, vocabulary headwords, and ordinary reading or dialogue text uncluttered. Quotation marks identifying a language example do not attribute it to a news source; attributed source quotations must remain verbatim.

Practice setting: B2 independent. Use natural professional English with brief explanations. Let me attempt each task without a model first. Offer a hint after difficulty and invite a more precise retry.

REFERENCE

Course: Medical Laboratory Technician English

Professional audience: Medical laboratory technicians and clinical laboratory support staff

Scope: All specimens, identifiers, incidents, times, and results are fictional. This curriculum teaches English, not collection, testing, instrument operation, biosafety, transport, clinical interpretation, diagnosis, or treatment. Module 7 supplies invented analytes, values, and units solely for complete notification and read-back practice. Each critical designation belongs only to the fictional laboratory; no universal threshold is implied. Identity and recipient verification are stated facts, not a complete verification procedure. Duties depend on qualifications and local rules. Real critical results and hazards require the laboratory's actual immediate response, documentation, and escalation procedures.

Lesson 01: Accessioning and unambiguous identifiers

Original fictional case: The receiving log lists accession A314; a clinic caller asks about A341. No identity match has been established. Technician Noor can check the accession log. The caller must identify the requested case through the laboratory's privacy and verification arrangements.

Language workshop: Resolve the reference before answering the status question

Communication goal: Clarify similar laboratory identifiers without merging cases.

Language guidance: Repeat the complete identifier and name its source. A similar name, location, or code is not a verified match. These exchanges do not replace the laboratory's identity requirements.

Useful frames (complete the gaps with case facts): Your enquiry says ____; the receiving log lists _____. / Could you read the complete _____ back? / We have not yet established that _____ refers to _____.

Vocabulary: accession number: A laboratory-assigned identifier linking a received specimen or case to its testing records. / requisition: The clinician's paper or electronic request identifying the laboratory tests ordered. / specimen label: The identifying information attached to a specimen container under the laboratory's requirements. / accessioning: The laboratory's process of registering a specimen or case and

associating it with its order.

Speaking task: Prepare for two minutes. Speak for 45-60 seconds using the case facts, then respond to your partner's question. Switch roles and repeat without reading the model response.

Partner: Clinic caller: your note says A341; check the actual request when asked.

Guided response task: Complete a workplace dialogue by selecting one of four supplied options. Compare the feedback and say the completed exchange aloud. Do not write an open-ended message.

Optional second-round challenge: Say both cases came from the same clinic and ask whether that is enough to match them.

Model for comparison AFTER my attempt, not a response to give me first: Before we discuss a specimen status, could we clarify the accession number? Our receiving log lists A314, while your enquiry says A341. Those are different identifiers. Please check the reference on your request through our verification arrangements. I will keep the enquiries separate until we establish which case you mean; no match has been confirmed yet.

END REFERENCE

TASK: Choose grammar that preserves the meaning

1. Use the lesson's language workshop, or one actual sentence from the dialogue, to explain a single grammar distinction in no more than 90 words. Show how it affects timing, certainty, responsibility or politeness.
2. Give one brief occupational exchange containing a gap and four complete options A-D. Supply enough context for exactly one answer. Keep distractors plausible; distinguish grammar errors from wording that changes the facts. Stop and wait.
3. Explain why each choice fits or fails, then ask me to read the completed exchange aloud. Do not request a draft, original written sentence or essay.
4. Repeat with three new exchanges, one at a time, moving from recognition to a more subtle meaning contrast. Do not copy the published checks. Label invented variations and do not turn uncertainty into a promise.
5. End with a short contrast table and two supported spoken turns using supplied frames. Accept accurate alternatives in oral practice; acknowledge and replace ambiguous multiple-choice items.

END PROMPT

Choose a clear message

START PROMPT / COPY THROUGH END PROMPT | ONLINE VERSION

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Partner: Clinic caller: your note says A341; check the actual request when asked.

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END REFERENCE

TASK: Coach guided workplace message choices

1. Use only the supplied reference and create one short, realistic dialogue with professional role labels and exactly one missing word or phrase. Supply four distinct options labelled A-D. Ensure that exactly one option fits both the grammar and the stated facts. Do not request a draft, an essay, or any open-ended writing. Stop and wait for my choice.
2. Use plausible distractors based on word combinations, certainty, timing, responsibility, or register. Avoid obviously absurd options. Do not introduce an unverified diagnosis, approval, deadline, charge, or legal rule. If extra fictional facts are necessary, state them explicitly before asking the question.
3. After I choose, identify the correct option and explain briefly why each of the other three options does not fit this exchange. Quote the relevant case fact or wording. Invite me to read both roles aloud; do not claim to assess pronunciation from a typed response.
4. Offer another four-choice exchange that practices a different expression. Present only one question at a time, wait for my answer, and vary the correct letter. If more than one answer is defensible, acknowledge the ambiguity and replace the question rather than marking a reasonable choice wrong.
5. After four exchanges, recap two useful word combinations and one distinction I should revisit. Ask which supported scenario I want to rehearse next, then stop and wait.

END PROMPT